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Case Number: 25CHCV03171 Hearing Date: June 24, 2026 Dept: F43

Dept. F43

Hearing Date: 06-24-26

Case # 25CHCV03171, Sherman, et

al. v. Young, M.D., et al.

Trial Date: None set.

DEMURRER AND MOTION

TO STRIKE

MOVING PARTY: Defendant Charles A. Young, M.D.

RESPONDING PARTIES: Plaintiffs Jennifer Sherman and Anthony

Sherman

RELIEF REQUESTED

Order sustaining demurrer the First Amended

Complaint's Second and Third Causes of Action.

Order striking punitive damages from the

First Amended Complaint.

RULING: Demurrer is sustained with leave to

amend. Motion to strike is granted.

SUMMARY OF ACTION

Plaintiffs Jennifer Sherman and Anthony

Sherman (Plaintiffs) filed this medical malpractice action against defendants

Charles A. Young, M.D. and Henry Mayo Newhall Memorial Hospital on September 5,

2025. Plaintiffs allege that plaintiff

Jennifer Sherman sustained an ankle fracture due to defendant Young's negligent

conduct while he performed a surgery. Plaintiff

filed a First Amended Complaint on April 22, 2026, asserting (1) medical

malpractice against all defendants; (2) medical battery against defendant

Young; (3) intentional concealment against all defendants; and (4) loss of

consortium against all defendants.

Plaintiffs allege the following: On February 15, 2025, plaintiff Jennifer

Sherman presented to Henry Mayo's emergency department after an ice-skating

incident in which she sustained an acute comminuted, displaced proximal fibular

fracture, mildly displaced oblique, distal tibia fracture, and possible

non-displaced distal tibial fracture.

(FAC ¶ 12.) Plaintiff Jennifer

Sherman consented to a tibia-fibula ORIF procedure which defendant Young completed. (Id. ¶ 13.) Defendant Young's post-operative notice states "a posterior malleolus fracture" was identified on the distal ankle and "fixated using 2.4 mm cannulated screws. 2 roughly parallel K wires were placed under lateral fluoroscopic imaging, avoiding joint penetration. Measurements were taken after wires were inserted. The near cortex was drilled, and a short threaded 2.4 mm cannulated screw was placed over each wire. This provided stabilization of the posterior malleolus fracture." (Id. ¶ 13.) The ankle fracture was mentioned in no other document, and at no time after the procedure did Defendants disclose to Plaintiff that (1) an ankle fracture had occurred intraoperatively, or (2) surgical fixation of the ankle had been performed. (Id. ¶ 14.)

Plaintiffs allege plaintiff Jennifer Sherman was not provided with complete medical records and radiographic imaging concerning the ankle-related fracture or diagnosis which prevented her from discovering the true nature and cause of her injury. (FAC ¶ 15.)

The ankle injury required longer hospitalization and continues to require both surgical and non-surgical care and remains painful. (Id. ¶ 18.) Plaintiffs assert that plaintiff Jennifer Sherman did not arrive at Henry Mayo with an ankle fracture and that the fracture was caused by the negligent and unnecessarily forceful and violent conduct defendant Young performed during the ORIF procedure. (Id. ¶ 16.)

On May 22, 2026, defendant Charles A. Young,

M.D. (Defendant) filed the instant demurrer and motion to strike. On June 8, 2026, Plaintiffs filed oppositions. On June 12, 2026, Defendant filed a

reply.

MEET AND CONFER

Demurrers and motions to strike must be

accompanied by a "meet and confer" declaration stating that the parties met and

conferred "in person, by telephone, or by video conference" and stating that

(1) the parties failed to reach an agreement resolving the issues raised in the

demurrer and motion to strike; or (2)

that the party who filed the pleading subject to the demurrer or motion to

strike failed to respond to the meet and confer request or failed to meet and

confer in good faith. (Code Civ. Proc.,

§§ 430.41, subd. (a)(3), 435.5, subd. (a)(3).)

In Department F43, meet and confer means in person or via phone, not by

letter or email. (Department F43

Courtroom Information, p. 2.)

The parties never met and conferred

directly. (Declaration of Kevin P.

Miller ¶ 3.) However, the court

exercises its authority to consider the merits of the moving papers.

SUMMARY OF ARGUMENTS

Defendant contends Plaintiffs fail to allege

sufficient facts demonstrating Defendant exceeded the scope of consent because

there was no procedure performed on the distal fibula described in the

complaint. There is confusion concerning

the tibia versus fibula. Moreover,

Plaintiffs fail to allege that plaintiff Jennifer relied on a concealed fact

and suffered any harm as a result of a concealed fact, specifically that

Defendant aggressively mishandled her ankle during surgery. Even though the medical records may have

failed to disclose any fact after the occurrence of the fracture, is irrelevant

because Plaintiff could not reasonably have relied on a medical record prepared

and obtained after the fact to her detriment.

Plaintiff's alleged harm from the ankle fracture itself is a separate

and distinct injury from the injury caused by the supposed concealment of the

fracture. Finally, Defendant contends

the court should strike punitive damages because this is a professional

negligence matter which requires leave of the court before making a claim for

punitive damages.

Plaintiffs have attached a proposed Second

Amended Complaint to correct the drafting error concerning the anatomical

description of the fracture and eliminates any ambiguity concerning the fracture that was allegedly caused during the surgery. As for the intentional concealment claim, Plaintiffs asserts the claim is based on the failure to disclose after surgery that the ankle fracture occurred during the ORIF procedure and required additional surgical procedures to repair it.

According to Plaintiffs, defendants concealed these material facts by omitting them from post-operative documentation and continuing to characterize plaintiff Jennifer Sherman's condition as merely a tib-fib fracture. This deprived plaintiff Jennifer Sherman of the ability to make informed medical and legal decisions concerning her care. As for the motion to strike, Plaintiffs claim the allegations concerning punitive damages do not request punitive damages but states Plaintiffs' intention to the request punitive damages upon a motion to the court.

REQUESTS FOR JUDICIAL NOTICE

Defendant asks the court to take judicial notice of the verifiable scientific fact that "posterior malleolus" is part of the back of the tibia bone and not part of the fibula, pursuant to Evidence Code section 452, subdivision (g).

Defendant cites two sources to support the request: (1) (IMAIOS e-Anatomy, "Posterior Malleolus, <https://www.imaios.com/en/e-anatomy/anatomical-structures/posterior-malleolus-1537020496#>);

and (2) (Bourne & Sinkler, Anatomy,

Bony Pelvis and Lower Limb: Tibia (StatPearls, updated July 24, 2023) NCBI

Bookshelf "<https://www.ncbi.nlm.nih.gov/books/NBK526053/>."

The court grants the request as follows: The posterior malleolus is part of the back

of the tibia bone and is not part of the fibula.

ANALYSIS

A. Demurrer

A party may respond to a pleading against it by demurrer

based on one or more of eight enumerated grounds, including that "the pleading

does not state facts sufficient to constitute a cause of action" and is

uncertain, meaning "ambiguous and unintelligible." (Code Civ. Proc., § 430.10, subds. (e),

(f).) The grounds for demurring must be

apparent from either the face of the complaint or a matter of which the court

may take judicial notice. (Code Civ.

Proc., § 430.30, subd. (a); see also *Blank v. Kirwan* (1985) 39 Cal.3d

311, 318.)

The purpose of a demurrer is to challenge the sufficiency

of a pleading "by raising questions of law."

(*Postley v. Harvey* (1984) 153 Cal.App.3d 280, 286.) "In the construction of a pleading, for the

purpose of determining its effect, its allegations must be liberally construed,

with a view to substantial justice between the parties.” (Code Civ. Proc., § 452.) The court “treat[s] the demurrer as admitting

all material facts properly pleaded, but not contentions, deductions or

conclusions of fact or law[.]” (Berkley

v. Dowds (2007) 152 Cal.App.4th 518, 525.)

In applying these standards, the court liberally construes the complaint

to determine whether a cause of action has been stated. (Picton v. Anderson Union High School

Dist. (1996) 50 Cal.App.4th 726, 733.)

1. Second Cause of Action: Medical Battery

Medical battery arises when a physician performs a

medical procedure without the patient’s consent or in deliberate deviation from

the consent given. To establish medical

battery, plaintiff must allege (1) consent was conditional, (2) a doctor

intentionally violated the condition while providing treatment, and (3) the

patient suffered harm as a result of the doctor’s violation of the

condition. (Conte v. Girard

Orthopaedic Surgeons Medical Group, Inc. (2003) 107 Cal.App.4th 1260,

1269.) “[A] physician who performs a

medical procedure without the patient’s consent commits a battery irrespective

of the skill or care used.” (Id.

at pp. 1266-1267.)

Plaintiffs allege plaintiff Jennifer consented to a

“tibia and fibula ORIF procedure,” a procedure that did not involve or require

a fracture to her ankle. (FAC ¶¶

28-29.) But Defendant’s violent

mishandling of plaintiff Jennifer’s lower extremities during the ORIF procedure

caused a fracture to her ankle and required Defendant to perform a more

extensive surgical procedure to which Plaintiff never consented. (Id. ¶ 29.) Defendant performed a distinct and separate

procedure involving the ankle (fixation of the posterior malleolus with screws

and K-wires) but did not disclose this to plaintiff Jennifer and was not

authorized to perform the surgery by plaintiff Jennifer. (Id. ¶ 28.) Plaintiff claims the ankle fracture and

surgical repair to the ankle have caused her to suffer and continue to suffer

pain and require both surgical and non-surgical treatment.

Defendant asserts the claim fails in part because the

posterior malleolus is part of the distal tibia rather than the fibula. Plaintiff claims she arrived at Henry Mayo

with a distal tibia fracture and possible non-displaced distal tibial fracture

but that no radiographic or other evidence demonstrates she arrived at the

hospital with a distal fibula—fractured ankle.

(FAC ¶ 12.) After taking judicial

notice of the posterior malleolus fact and reviewing paragraph 12 along with

the allegations in the Second Cause of Action (Id. ¶¶ 28-33), the court

finds that Plaintiffs' allegations concerning the scope of consent are internally inconsistent, considering Plaintiffs also allege plaintiff Jennifer consented to a "tibia and fibula ORIF procedure" and that the "additional" posterior malleolus procedure would have involved the tibia bone rather than the fibula.

Thus, Plaintiffs do not clearly allege Defendant caused the ankle fracture during surgery and in turn went beyond the scope of informed consent to repair the ankle fracture.

Plaintiffs present a copy of a proposed second amended complaint which clarifies the bones that form the ankle and that Defendant's operative note did not address certain bones (lateral malleolus/distal fibula) which were supposedly repaired during the "additional" surgery.

Therefore, the court sustains the demurrer to the First

Amended Complaint's Second Cause of Action with leave to amend.

2. Third Cause of Action: Intentional Concealment

To establish fraudulent concealment, the complaint must plead "(1) concealment or suppression of a material fact; (2) by a defendant with a duty to disclose the fact to the plaintiff; (3) the defendant intended to defraud the plaintiff by intentionally concealing or suppressing the fact; (4) the plaintiff was unaware of the fact and would not have acted as he or she

did if he or she had known of the concealed or suppressed fact; and (5) plaintiff

sustained damage as a result of the concealment or suppression of the

fact.” (Graham v. Bank of America,

N.A. (2014) 226 Cal.App.4th 594, 606.)

Fraud claims must be pled with specificity; general and conclusory

allegations do not suffice. (Lazar v.

Superior Court (1996) 12 Cal.4th 631, 645.)

The complaint must plead facts showing “how, when, where, to whom, and

by what means the representations were tendered.” (Ibid.) The facts must be clear enough to inform the

opposing party what they must answer.

(See Scafidi v. Western Loan & Building Co. (1946) 72

Cal.App.2d 550, 558.) “[C]onclusionary

allegations that the omissions were intentional and for the purpose of

defrauding and deceiving plaintiffs and bringing about the purchase . . . and

that plaintiffs relied on the omissions in making such purchase are

insufficient [to show fraud by concealment].”

(Goodman v. Kennedy (1976) 18 Cal.3d 335, 347.)

“[A] duty to disclose arises in this context only where

there is already a sufficient relationship or transaction between the

parties.” (Bigler-Engler v. Breg,

Inc. (2017) 7 Cal.App.5th 276, 312.)

A sufficient relationship arises when a transaction occurs between the

parties such as that of a buyer and a seller, an employer and a prospective

employee, or parties entering into any kind of contract. (LiMandri v. Judkins (1997) 52

Cal.App.4th 326, 337.) “[N]ondisclosure

or concealment may constitute actionable fraud:

(1) when the defendant is in a fiduciary relationship with the

plaintiff; (2) when the defendant had

exclusive knowledge of material facts not known to the plaintiff; (3) when the defendant actively conceals a

material fact from the plaintiff; and

(4) when the defendant makes partial representations but also suppresses some

material facts.” (LiMandri v. Judkins (1996) 52 Cal.App.4th 326,

336 [quoting Heliotis v. Schuman (1986) 181 Cal.App.3d 646, 651]; see

also Warner Constr. Corp. v. City of Los Angeles (1970) 2 Cal.3d

285, 294.)

Here, a special relationship existed between Defendant, a

medical professional, and Plaintiff, his patient. (Alexander v. Scripps Memorial Hospital La

Jolla (2018) 23 Cal.App.5th 206, 235 [Physician-patient relationship giving

rise to duty of care exists where doctor affirmatively treats and advises

patient.])

Plaintiffs allege defendants concealed the material fact

that plaintiff Jennifer sustained an ankle fracture during the ORIF procedure

and that surgical fixation of the fracture was performed. (FAC ¶ 41.)

Although Defendant's operative note references and identifies fixation

of a posterior malleolus fracture, no other medical records or post-operative

notes disclose the existence of an ankle fracture, referring only to a

tibia-fibular injury. (Id. ¶ 41.)

As mentioned above, the First Amended Complaint is

internally inconsistent concerning whether the fractures plaintiff Jennifer

originally presented with were tibia fractures or fibula fractures. This is important because the additional

surgery addressed a fracture in the posterior malleolus which includes the tibia

but not the fibula. Yet, Plaintiffs

allege plaintiff Jennifer consented to a "tibia and fibula ORIF procedure." (FAC ¶ 12.)

Thus, it is unclear whether Defendant did conceal the

fact of the ankle fracture and that the repair would occur as part of the

procedure to which plaintiff Jennifer consented. Furthermore, it is unclear how the material

fact was concealed when Plaintiffs allege the "posterior malleolus" repair was

revealed in the operative note and Plaintiffs do not state how they obtained

the note.

Plaintiffs do adequately allege that plaintiff Jennifer

suffered harm as a result of the surgery.

But the uncertainties in the First Amended Complaint make it difficult

for the court to determine whether plaintiff Jennifer actually relied on

Defendant's silence and whether Defendant intentionally hid the "additional"

surgery from her since it is mentioned in his operation note.

Therefore, the court sustains the demurrer to the First

Amended Complaint's Third Cause of Action with leave to amend, as to defendant

Charles A. Young, M.D.

B. Motion to Strike

"Any party, within the time allowed to respond to a

pleading may serve and file a notice of motion to strike the whole or any part

thereof." (Code Civ. Proc., § 435.) A court may, upon motion or at any time,

strike from the complaint "any irrelevant, false, or improper matter" or "all

or any part of any pleading not drawn or filed in conformity with the laws of

this state, a court rule, or an order of the court." (Code Civ. Proc., §§ 436, subds. (a)-(b),

187; *Greshko v. County of Los Angeles* (1987) 194 Cal.App.3d 822, 830

[noting the court's inherent power to limit the focus of the proceedings to the

issues framed by the remaining viable pleadings].) An "irrelevant matter" is an allegation that

"is not essential to the statement of a claim or defense," "is neither

pertinent to nor supported by an otherwise sufficient claim or defense," or "a

demand for judgment requesting relief not supported by the allegations of the complaint or cross-complaint.” (Code

Civ. Proc., § 431.10, subds. (b)-(c).)

The court must read the factual allegations in the complaint as a whole and assume their truth. (Clauson v.

Superior Court (1998) 67 Cal.App.4th 1253, 1255.)

Defendant Young moves to strike punitive damages from the

First Amended Complaint:

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Paragraph 50 on Page 12: “Upon motion to the court, and granting

thereof, Plaintiff will seek exemplary damages in the within action in a sum according to proof.”

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Paragraph 2 of the Prayer for Relief on Page

13: “Upon motion to the court, and the granting thereof, for punitive or

exemplary damages where appropriate in an amount to be determined at trial.”

1. Professional Negligence and Punitive Damages

A motion to strike punitive damages may lie where the

alleged facts do not rise to the level of “malice, fraud, or oppression”

required to support punitive damages. (Turman

v. Turning Point of Central California, Inc. (2010) 191 Cal.App.4th 53,

63.) To state a prima facie claim for

punitive damages, a complaint must set forth specific facts demonstrating the elements

stated in Civil Code section 3294. (College

Hospital, Inc. v. Superior Court (1994) 8 Cal.4th 704, 721; see also Brousseau v. Jarrett (1977)

73 Cal.App.3d 864, 872.)

“[A]ny claim for punitive damages in an action against a

health care provider be subject to [Civil Code section 425.13] if the injury

that is the basis for the claim was caused by conduct that was directly related

to the rendition of professional services.”

(Central Pathology Service Medical Clinic, Inc. v. Superior Court

(1992) 3 Cal.4th 181, 192-193 [“Plaintiffs’ cause of action for fraud in this

case is directly related to the manner in which defendants provided

professional services. The

claim emanates from the manner in which defendants performed and

communicated the results of medical tests, a matter that is an ordinary and

usual part of medical professional services.”].)

“In any action for damages arising out of the

professional negligence of a health care provider, no claim for punitive

damages shall be included in a complaint or other pleading unless the

court enters an order allowing an amended pleading that includes a claim for punitive damages to be filed. The court may allow the filing of an amended pleading claiming punitive damages on a motion by the party seeking the amended pleading and on the basis of the supporting and opposing affidavits presented that the plaintiff has established that there is a substantial probability that the plaintiff will prevail on the claim pursuant to Section 3294 of the Civil Code. The court shall not grant a motion allowing the filing of an amended pleading that includes a claim for punitive damages if the motion for such an order is not filed within two years after the complaint or initial pleading is filed or not less than nine months before the date the matter is first set for trial, whichever is earlier." (Civ. Code, § 425.13, subd. (a).)

Here, Plaintiffs cite no statutory authority or case law which supports the proposition that a plaintiff in a professional medical negligence matter may include a conditional or anticipatory claim for punitive damages or placeholder allegations reserving the right to seek exemplary damages upon future motion in the operative pleading.

Absent a good faith basis for modifying or extending an existing law, litigants are generally prohibited from asserting a position in litigation without authority. (See, e.g., *In re Estate of Randall* (1924) 194 Cal. 725, 728-729 ["Contentions supported neither by argument nor by

citation of authority are deemed to be without foundation, and to have been abandoned.”]; see also Lafferty v.

Wells Fargo Bank (2013) 213 Cal. App. 4th 545, 571-572 [“When a point is asserted without argument and authority for the proposition, ‘it is deemed to be without foundation and requires no discussion by the reviewing court.”]; Atchley v. City of Fresno

(1984) 151 Cal. App. 3d 635, 647 [“Where a point is merely asserted by appellant’s counsel without any argument of or authority for the proposition, it is deemed to be without foundation and requires no discussion by the reviewing court.”].)

Although Plaintiffs base their claims in allegations of professional medical negligence, no such motion for leave to include punitive damages has been filed.

Therefore, the court grants defendant Charles A. Young,

M.D.’s motion to strike punitive damages from the First Amended Complaint.

CONCLUSION and ORDER

Demurrer to the First Amended Complaint’s

Second and Third Causes of Action is sustained with leave to amend. This ruling applies only to defendant Charles

A. Young, M.D.

Motion to strike is granted. The court strikes the following from the

First Amended Complaint: Lines 17-18 on

Page 12 and Lines 19-20 on Page 13.

Plaintiffs may file and serve an amended complaint within

twenty (20) days of this order.

Plaintiffs to give notice.